Worth County, Iowa

Continue routine review

Gap review score **7.6** / 100

Why it surfaced

Worth County has a published gap review score of 7.6, placing it in the Continue routine review category. The county has no county-level published Lyme disease case, tick, or pathogen records in the current datasets. Local information may help interpret these gaps. This score is not a measure of individual risk.

County data: Six review factors

These factors summarize the published data used in the Atlas. They do not show how much each factor contributed to the county's overall score.

1	**Published Lyme case data**	No county-level 2023 Lyme disease case count was published. A missing record is not treated as zero cases. 77 published 2023 human-surveillance records were not allocated to a county in this release. Check whether any belong in this county before interpreting the county figure.
2	**Tick presence**	The published tick table has no county record; this does not establish tick absence.
3	***Borrelia burgdorferi* detection**	No county-level *Borrelia burgdorferi* detection record is available; this does not establish pathogen absence.
4	**Social vulnerability**	The 2022 social vulnerability percentile is 2.3. Higher values indicate greater vulnerability on the source index. This is context for access and outreach, not proof of underreporting.
5	**Health insurance coverage**	The published percentage uninsured is 5.9%. This is context for healthcare access, not proof that cases are being missed.
6	**Rurality**	USDA Rural-Urban Continuum Code 9 (2023): completely rural or less than 2,500 urban population, not adjacent to a metro area. Code 1 is the most metropolitan and 9 is the most rural. This is context for follow-up planning, not a tick-habitat or underreporting measure.

Interpreting the data

Lyme disease data are grouped by county of residence, not necessarily the location of exposure. These values describe published surveillance, not individual risk. A missing published case count is not an observed zero. A missing tick record does not establish that ticks are absent. Missing pathogen data is not a documented negative finding. The percentage uninsured is context; the score uses the uninsured percentile.

Observation periods: Lyme case records 2023; social vulnerability 2022; rurality 2023.

Suggested follow-up

Use existing surveillance and program processes. Do not initiate an investigation solely because this county is scored.

1	**Human disease surveillance**	Continue routine review of Lyme disease surveillance data through established program processes. Check whether additional county case information is available through local records, including suppressed or unallocated reports. A missing published count is not a finding of zero cases.
2	**Vector and pathogen surveillance**	Review new tick and pathogen information as part of established program activities. Record gaps in published coverage without interpreting missing records as absence or initiating sampling solely because data are unavailable. Determine whether records exist outside the current source.
3	**Clinical and laboratory reporting**	Maintain awareness of existing reporting and laboratory surveillance procedures. Consider whether routine reporting updates warrant review. This is not a finding that laboratories failed to report.
4	**Community context and outreach**	Continue existing prevention education and outreach activities as appropriate. Use social vulnerability, health insurance coverage, and rural-urban classification as context for access and outreach. These measures are not proof of transmission or underreporting.
5	**Coordination and documentation**	Document the published data, any new information, and key limitations during routine program review. No additional investigation is required based on the current score.

Questions before acting

1. Are current surveillance processes sufficient to review this county as new data become available?
2. Are gaps in the published human or vector evidence best documented for future review?
3. Do available local records suggest a reason to depart from established monitoring practices?
4. Are current prevention and communication activities appropriate for the community context already known?

Data sources and more information

Definitions, data sources, and methods are available on the Atlas website.

Lyme surveillance data | Tick and pathogen data | Clinical testing guidance | Social vulnerability index | Health insurance data | Rural-urban classification | County review methodology | MRLC Annual NLCD 2025 reviewed demo cohort (C1V2-2025) | USDA ERS Rural-Urban Continuum Codes (2023) | CDC/ATSDR Social Vulnerability Index (2022 (2018-2022 ACS)) | CDC Lyme surveillance (2023)

Document details

County: Worth County, Iowa
County identifier (FIPS): 19195
Published gap review score: 7.6
Review category: Continue routine review

Data reference years

Lyme case records: 2023
Social vulnerability: 2022
Rurality: 2023

Document information

Report date: 2026-10-21
Template version: county data brief v3.0
Website: onehealthatlas.org
Data release: governed-2026-10-15-january-climate-prod

Data methodology: semantic-2.0.0
Score methodology: surveillance-gap-score-v1.1; smooth k=50
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Source vintages: MRLC Annual NLCD 2025 reviewed demo cohort (C1V2-2025); USDA ERS Rural-Urban Continuum Codes (2023); CDC/ATSDR Social Vulnerability Index (2022 (2018-2022 ACS)); CDC Lyme surveillance (2023); CDC ArboNET tickborne pathogen status (through 2025-12-31); CDC ArboNET county tick status (through 2025-12-31)
Published release only. What-if score settings are not applied.
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